

Correspondence.

THE ELECTION OF DIRECT REPRESENTATIVE.

SIR,—I wish to thank the medical practitioners of England and Wales for choosing me to be their direct representative on the General Medical Council at the recent election rendered necessary by the regretted death of Dr. McManus. Immediately after the result of the election was declared I was called upon to attend the session of the Council at which the National Insurance Bill was discussed, and the report upon certain of its medical aspects prepared, which drew from the Chancellor of the Exchequer the reply published last week.¹ The consideration of the bill by the British Medical Association in the Representative Meeting and the Special Committee has made such great inroads on time and energy that my friends will, I am sure, forgive the delay in returning my thanks; though tardily expressed my gratitude is none the less sincere.

It shall always be my endeavour to justify the confidence my brother practitioners have shown by electing me to so responsible a position at this critical time.—I am, etc.,

Taunton, June 26th.

J. A. MACDONALD.

STATE SANATORIUMS.

SIR,—Dr. Allison, in the BRITISH MEDICAL JOURNAL of June 17th, p. 1449, suggests a figure of £250 a bed as the cost to be allowed in the erection of State sanatoriums as against Mr. Lloyd George's proposal that the cost should not exceed £100. Our experience at the Maitland Sanatorium works out exactly halfway between these estimates, as we have accommodation for sixty patients, men, women, and children, at a total cost of £10,500, or £175 per bed. This includes the purchase or erection of:

- (1) A 40-acre farm with farm buildings.
- (2) Pavilions (some wood, some brick), furnished and equipped for adult patients and staff, with several sleeping shelters.
- (3) A children's house.
- (4) A fully-equipped steam laundry.
- (5) Acetylene and petrol gas installations.

Our buildings are mostly of a very simple character, and we have found it necessary to improve the type in the more recent additions. A State sanatorium could hardly be erected for as little as has been spent here, so that probably £200 a bed is quite the lowest figure that could be counted upon, however simple the arrangements were.

The possession of a farm and its equipment is an invaluable asset to a sanatorium from the point of view of "graduated labour." The cost of maintenance should not be reckoned at less than 30s. a week if the sanatorium scheme is to contain the principle of employing its ex-patients as members of the staff in every possible direction, becoming thus to some extent a labour colony as well as a sanatorium. I cannot speak too strongly of the importance of this as an integral part of sanatorium life: (1) The presence of apparently cured patients is a constant encouragement and stimulus to the others; (2) the opportunity offered to patients to test their working capacity, while still sheltered by the supervision of the sanatorium staff, speaks for itself; (3) the discipline and moral tone of the sanatorium is greatly helped by this body of non-commissioned officers between patients and authorities. They are able to educate the ordinary patients in innumerable ways not possible to nurse or doctor. On the other hand, the liability to breakdown, and the necessity for consequent overlapping arrangements, slightly raises the cost per head. At 30s. a week, however, the sanatorium can be maintained with very careful management and very little, if any, margin.—I am, etc.,

ESTHER CARLING,

Maitland Sanatorium,
Peppard Common, Oxon., June 26th.

Medical Superintendent.

THE PROVISION OF SANATORIUMS AND THE ENDOWMENT OF RESEARCH.

SIR,—The article on this subject published in the JOURNAL of June 10th, p. 1392, contains the following: "The sanatoriums have applied the knowledge provided by the schools of research," and infers that it is more especially on the work of Koch and the bacteriologists that sanatorium treatment is founded. Again, it is stated that

¹ SUPPLEMENT, June 24th, page 474.

"It is only by giving expert medical authorities the power of carrying on investigations that an advance towards the extinction of consumption can be hoped for."

First, we beg to ask, What schools of research provided Bodington, Brehmer, Otto Walther, and the other pioneers with the knowledge and acumen they applied when first they carried out that *common-sense* treatment of consumption which has revolutionized our treatment of the disease, and which is quietly revolutionizing medicine? It would be difficult to exaggerate the importance of Koch's great discovery, but his work and that of his followers were subsequent to the first advocacy of the principles of sanatorium treatment which are now so widely accepted.

Secondly, do we not already possess a knowledge of the methods by which the extinction of tuberculosis can be brought about? The educational influence of sanatoriums has a great preventive value. This, and such measures as the establishment of dispensaries, where it is of especial importance that arrangements for the free examination of sputum should be made, give good hope of extinguishing tuberculosis, now that means are being found for carrying out the methods advocated. It is of great importance, therefore, at the present juncture, that authoritative medical pronouncements should indicate the proper distribution of the means provided.

We would not in the least detract from the great value of past researches, nor from the advisability of pursuing further investigations, but one of the most fortunate circumstances in this whole question is that the methods which are applicable for bringing about the recovery of the consumptive are largely identical with those proved to be most advantageous for prevention. We, therefore, cannot agree that research, important as it is and always must be, "can alone point the way to its prevention." The methods to use for prevention are already clear.—We are, etc.,

JANE WALKER.

F. W. BURTON-FANNING.

NOEL BARDSWELL.

S. VERE PEARSON.

June 12th.

THE VACCINATION DEBATE.

SIR,—In my letter of June 17th I pointed out that childhood above the age of 5 years was the least fatal period of life for small-pox, so that for this age-period the need for vaccination was not very urgent. Dr. Cameron, however, wishes me to consider the case of very young children and especially infants. I admit that, in the case of the latter, small-pox, when it does attack, is very serious, but my point is that the risk of attack in infants is comparatively small—judging, at least, from the experience of Leicester. In the last twenty-eight years, since vaccination was practically abandoned in Leicester, there have been only 5 deaths of infants from small-pox and 10 deaths in the age-period 1 to 5 years, surely not a very heavy penalty to pay when it is remembered that, on the other side of the balance-sheet, considerably over 100,000 infants have escaped the risks of vaccinia. I have no wish to exaggerate these risks, but in the aggregate they are very far from being a negligible quantity. Moreover, I would point out that in any case compulsory vaccination does not now protect the first six months of life—and never protected the first three months—so that it would not be fair to attribute all the small-pox deaths in infancy to neglect of vaccination.

Now, I am far from being alone in thinking that in the case of infants and very young children compulsory vaccination is not very urgent. Some of your readers may not be aware that five years ago the Honorary Secretary of the Jenner Society (Dr. F. T. Bond) and the Honorary Secretary of the Imperial Vaccination League (Mrs. Garrett Anderson) wrote a joint letter to the *Times* (April 26th, 1906), in which they admitted that there was

a growing opinion that, in consequence of altered social conditions and improved sanitary administration, it is not absolutely necessary to have infants of a few months old vaccinated, except in the presence of epidemic small-pox;

and they therefore advocated the postponement of vaccination until school age. There is a great deal to be said for this proposition, and I am glad to range myself alongside such stalwart provaccinists in advocating it. In view,

however, of the fact mentioned above that the age-period 5 to 20 years is the least fatal period of life as regards small-pox, I would go further than Dr. Bond and Mrs. Garrett Anderson and suggest that the age limit might with reasonable safety be raised to 20 years, and at that age, according to Dr. Cameron, there would be no harm in allowing people to choose for themselves. As a matter of fact, there is a perfectly sound argument in favour of such a proposal. Small-pox is essentially a disease which is spread by adults rather than by children; at least this has been our experience in Leicester. Where children fall victims to it it is usually because they have been infected by adults. Hence the remarkable immunity of the elementary schools in Leicester. It is therefore the adults rather than the children that really need the protection afforded by vaccination. But it is agreed that infantile vaccination is useless, so far as protection from attack is concerned, after twenty years (in my view it is often worse than useless, because of the danger of "missed" cases), so that if vaccination is only going to be performed once in a lifetime—and we all admit that universal revaccination is out of the question—the ideal period would be in adolescence rather than infancy. Moreover, primary vaccination at that period of life would probably be more lasting than when performed in infancy.

Dr. Stansfield seems to think that because I believe in vaccination and use emergency vaccination as part of the "Leicester method" it is an argument in favour of compulsory infantile vaccination. I am unable to follow his reasoning here, so must pass the point; but I may say that he is quite in error when he asserts that when small-pox breaks out in Leicester "nearly all the inhabitants go off in hot haste to get vaccinated." It would have been better if Dr. Stansfield had verified this point before using it as an argument. I speak from knowledge when I say that in Leicester, apart from the better-class residential districts, the amount of vaccination during an epidemic is almost negligible. I had a special house-to-house inquiry made at the close of the 1903 epidemic, and I found that, excluding the houses actually attacked, only 3 per cent. of the inhabitants had been vaccinated since the epidemic began. In the 1904 epidemic the amount of vaccination performed was even less. It can hardly be suggested that this small amount of vaccination could have played any appreciable part in bringing the epidemic to a close. Dr. Stansfield must understand that in Leicester we do not try to make a "scare," when a few cases of small-pox occur, in order to frighten people into getting vaccinated. The putting up of startling posters, magnifying the fact that a case of small-pox has occurred, and urging people to get vaccinated, is emphatically not part of the Leicester method, and, incidentally, it is apt to do serious injury to trade. It is methods such as this which tend to make an outbreak of small-pox a much more expensive matter in many towns than is the case in Leicester. Dr. Stansfield falls back, as so many have done before him, upon the experience of Gloucester as a set-off against that of Leicester. This is most unfair. Will any one seriously suggest that the methods which have proved so successful in Leicester were fairly tried at Gloucester? It is notorious that they were not, so that the unfortunate experience of Gloucester proves nothing as to the adequacy of those methods.

I am quite prepared to admit this, however. If in any community at any time small-pox really got out of hand, I should certainly advise that, as a last resort, wholesale vaccination of the general population, at least in the affected districts, should be immediately carried out. Given a free hand, and in the absence of the present bitter prejudice against vaccination, engendered by compulsion, such a general vaccination, as an emergency measure, should be quite feasible, and would cut short any epidemic in a few weeks. But with increasing public health efficiency throughout the country, the necessity for such a drastic step should very rarely, if ever, arise. Nevertheless, it could be done (it has been done before now), and it constitutes, if I may say so, the trump card of those of us who, whilst we believe firmly in vaccination, do not believe in the necessity for our present irritating, inefficient, and half-and-half system of compulsory vaccination. Provided the people of Leicester, or of any other town which neglects infantile vaccination, will get vaccinated if and when the necessity for it really occurs, they

have no need to fear being decimated by small-pox, in spite of Dr. Stansfield's gloomy forebodings. I am aware that there is this present danger; the "man in the street" has so often been urged to get vaccinated when no real necessity for it existed, as shown by subsequent events, that he has to a large extent lost faith in medical advice on the question of vaccination. This, however, would right itself under the new régime, and, vaccination only being advised when really necessary, neglect to follow medical advice would bring its own punishment, which at present is certainly not the case.

In concluding this letter, Sir, allow me to thank you for according me so much of your valuable space. This question of vaccination is not so simple as many medical men believe. So much time has been lost in debating it with those who disbelieve in it—who regard the whole thing as a vast delusion—that the issues now being raised have hitherto not been discussed, or only very inadequately. It is desirable that they should be discussed, and some one must assume the responsibility of challenging orthodox teaching. My only excuse for putting myself forward to do so is that in consequence of my position as Medical Officer of Health for Leicester, I have had an experience which is certainly exceptional, and probably unique.—I am, etc.,

C. KILICK MILLARD,

Leicester, June 24th.

Medical Officer of Health.

SIR,—Dr. Millard thinks we need not be very concerned if we have to abandon compulsory infantile vaccination, and I quite agree with him if it were made compulsory to vaccinate all suspects in case of an outbreak. I have been led to form that opinion (1) from my faith in vaccination; (2) from the strict quarantine regulations and the perfection of our public health authorities; (3) from the fact that the infant is apt to have diseases for which vaccination is blamed; and, lastly, because stopping infantile vaccination might perhaps pacify the most rabid anti-vaccinationists, whose clamour cannot be entirely disregarded.—I am, etc.,

Hawick, June 21st.

JOHN HADDON, M.D.

. While ready to receive any correction of an error of fact, we cannot undertake to continue this correspondence.

MEDICAL OFFICERS OF HEALTH AND OFFICIAL DUTIES.

SIR,—Among the advertisements in the BRITISH MEDICAL JOURNAL of June 24th will be found an invitation for applications for the post of medical officer of health and school medical officer for the borough of Hartlepool, at the salary of £250 per annum, to be increased by yearly increments of £10 to the maximum of £300. The successful candidate will not be allowed private practice, but will be allowed to hold any of the offices mentioned in Section 3 of the Memorandum of the Local Government Board, 1910, "if any applicant is successful in obtaining the same." That is to say, the officer appointed will be allowed to supplement his salary by means of such other extras as are afforded by the posts of Poor Law medical officer, public vaccinator, police surgeon, post office medical officer, and certifying factory surgeon.

The next Representative Meeting of the British Medical Association will be asked to confirm the resolutions of its Central Council in (1) That medical officers of health be confined to official duties; (2) that the system of part-time appointments (except that of medical officer of health) for general practitioners be maintained.

To us it seems that the two resolutions cannot possibly be compatible. Obviously if the medical officer of health be confined to "official duties" the "official duties" will be embraced by the various part-time offices held at present by men in general practice.

The inevitable result will be a system illustrated by the concrete example given above.

Now, Sir, it may fairly be asked, What will be the future policy of the British Medical Association? Will it support the recommendations of the Local Government Board, and favour medical officers of health getting all the official appointments so that they can obtain some approximation to a living wage? and if so, how will that policy reconcile itself to the other—that all part-time appointments, with